

Question:

A 34-year-old woman presents to her OB/GYN with complaints of missing her last 3 periods as well as intermittent spontaneous milky-white nipple discharge bilaterally for the past 3 months. Vital signs are stable and within normal limits. Neurologic examination is without abnormality, including normal visual fields. Serology and MRI of the brain are ordered, with results pending. Which of the following sets of laboratory results would be expected in this patient?

Answer Choices:

- A. Decreased prolactin, decreased FSH, decreased LH
- B. Decreased prolactin, increased FSH, increased LH
- C. Increased prolactin, decreased FSH, increased LH
- D. Increased prolactin, decreased FSH, decreased LH

Reasoning + Answer:

<reasoning process here> The patient is a 34-year-old woman presenting with absent periods (oligomenorrhea) and bilateral spontaneous milky-white nipple discharge. These symptoms are classic for hyperprolactinemia, which can cause galactorrhea and menstrual irregularities. Hyperprolactinemia leads to increased prolactin levels, which suppresses the hypothalamic-pituitary axis, resulting in decreased secretion of follicle-stimulating hormone (FSH) and luteinizing hormone (LH).

The milky nipple discharge is a hallmark of elevated prolactin, and the absence of periods is due to the suppression of FSH and LH, which are essential for ovulation and menstrual cycles. Therefore, the expected laboratory findings would be increased prolactin, decreased FSH, and decreased LH.

This pattern is consistent with primary hyperprolactinemia, which can be due to a pituitary adenoma or other causes, and is often confirmed with lab testing showing high prolactin and low FSH/LH.

Thus, the correct set of laboratory results is increased prolactin, decreased FSH, and decreased LH.

<answer> D. Increased prolactin, decreased FSH, decreased LH </answer>